

Mandate and ownership

Before you go • Before configuration starts

Name the six people whose support decides whether this works, speak to each separately, and put the agreement in writing.

Hospital	Unit / area	Completed by	Date

1. The six people — spoken to separately, by name

Role	Name	Met on (date)	Q1: What takes too long in your unit?	Q2: What would make this worth your staff's time?
Medical Director				
Head of Nursing				
Head of Pharmacy				
Laboratory Director				
Administrator / Finance				
IT Officer / technician				

If the IT Officer post does not exist, go back to B-2 — that is a stop-rule condition.

1b. Permission or instruction?

A director will say yes to visiting engineers. That yes changes nothing on the wards. The mandate is real only when the director tells staff, personally, that this is how the hospital will now work. This is the field that separates the sites that adopted from the sites that did not.

Did the director announce it to staff?	To whom	When	In what form (meeting / memo / word to each head)
Yes / No / Promised, not yet done			

If the answer is No, or if the director expects YOU to do the telling, you have permission and not a mandate. Say so here rather than proceeding as though you had one.

2. Super-user candidates (permanent staff, respected, curious — not the most senior)

Name	Unit	Why them	Time allowance agreed?

3. The written agreement (MoU) — tick when the clause exists in writing

- ☐ Who owns the system — the hospital, stated explicitly
- ☐ Who owns the data, and how patient consent is handled
- ☐ Training commitments: how many staff, released for how long, when
- ☐ Remote support: what the organisation will and will not do after leaving, and for how long
- ☐ Who pays for what after we leave: hosting, hardware replacement, the IT post
- ☐ Signed by the medical director

Existing systems discovered (finance package, any previous digital attempt, who ran it):
